



Policy

Clinical Alerts (SCGOPHCG)

Reference #: HP255

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	All Staff

Introduction

This policy details the requirements for clinical staff to ensure correct and complete reporting, notification and documentation of clinical / medical alerts (drug reactions, anaesthetic alerts, diet reactions and medical condition alerts).

The principles of this policy are based on the Department of Health MP0053/17 WA Clinical Alert (MedAlert) Policy, which mandates the implementation of a standardised process of communicating clinical alerts across the WA health system using the Patient Administration System (PAS).

The PAS clinical alert system must not be used for:

- Other PAS Alerts including Micro Alerts and patient and/or family member Behaviour (Risk) Alerts. These are outside the scope of, and are not defined within, the Clinical Alert Policy.
- MedicAlerts. The MedicAlert Foundation (who provides consumers with MedicAlert bracelets) has a registry to identify consumers with serious allergies or conditions and this is usually organised and authorised through the consumer's general practitioner. This system is outside the scope of the policy.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Definitions

Clinical Alert

A Clinical Alert is a diagnosis which has the potential to be of critical importance to a patient's management during the first 24 hours of their admission to hospital and assumes that the patient is not always capable of communicating such information.

There are three classifications of clinical alerts:

- a) Anaesthetic
- b) Medical condition
- c) Medication (drug)

Principles

- It is the responsibility of the treating specialist to report clinical alerts in accordance with specified medical, anaesthetic or drug-related alerts and specific detailing, as per the Med Alert/Clinical Alert Notification Form (MR002).
- It is the responsibility of clinicians to review clinical alert notifications in iSOFT Clinical Manager (iCM) and/or the patient's Healthcare record(s).
- The MR002 form must be used to add, update or delete a clinical alert.
- The SCGOPHCG Clinical Alert Committee is responsible for the governance of clinical alerts. The committee reports to the NSQHSS Communicable for Safety Standard Committee. Clinical alert reports will be sent to the SCGOPHCG Drug and Therapeutics Committee and SCGOPHCG Medication Safety Committee where appropriate.
- Authorised clerical staff may identify reported and approved (existing) clinical alert on PAS

Clinical Alerts that are deemed appropriate for inclusion onto the PAS are outlined on the MR002 form. Only serious life- threatening adverse drug reactions are to be documented on the PAS.

Both the drug implicated and the reaction which occurred must be specified. Refer to [Appendix 2](#) for serious adverse drug reactions for inclusion.

Procedure

The clinical alert process involves:

- Raising a clinical alert for a patient by completing an [Medical Alert/Clinical Alert notification form MR002 form](#). This form is to be completed by a member of the treating or consulting medical team (for all Alert categories) or pharmacist (for medication/drug related Alerts only).
- Assessing adverse drug reactions to determine whether they classify as a clinical alert – See [Appendix 2](#).
- Send completed form to PMI Office for the SCGOPHCG Clinical Alert Committee to review.
- Enter the Alert into the PAS after sending the MR002 Form to the PMI Office.
- If a drug-related clinical alert occurs during hospitalisation a patient must be provided with the [Adverse Drug Reaction Information Brochure for Consumers](#), or equivalent, that is completed by the treating medical team or pharmacist.
- Standardised documentation requirements for adverse drug reactions, which may, or may not, be elevated to serious drug reactions that require a clinical alert.

1.0 Identification of a Clinical Alert during admission

Responsibility	Emergency Department (ED)/inpatients	Outpatient areas
1. Communicate a potential clinical alert to the treating medical team if the patient meets the clinical alert criteria (where a clinical alert has not been generated by the treating team).	Nurse, RMO, Dietician (food related Clinical Alerts)	

Responsibility	Emergency Department (ED)/inpatients	Outpatient areas
2. Complete all mandatory fields of the electronic MR002 or paper form during downtime.	Consultant, Registrar, RMO, Clinical Pharmacist	
3. On completion of the electronic MR002, click the 'Submit' button to email a copy to: SCGH.PMIOffice@health.wa.gov.au	Consultant, Registrar, RMO, Clinical Pharmacist	
4. Print the completed MR002 form. Inpatients: Give printed MR002 form to ward clerk to file in the front of the patient's integrated progress notes. Outpatients: Attach form to the front of the patient's medical record and give to clerical staff. - Document the clinical alert in eProgress note and on the discharge summary (NaCS).	Consultant, Registrar, RMO, Clinical Pharmacist	
5. Affix ' Adverse Drug Reaction ' alert sticker on all current medication charts for ADRs and complete the allergy section.	All clinicians	
6. Notify nurse if an allergy or ADR has been identified	Consultant, Registrar, RMO, Clinical Pharmacist	Not required
7. Attach a red patient identification (ID) band to the patient if an allergy or ADR is identified.	Nurse	Not required
8. Complete allergy and ADR information on Hospital Medication chart	Nurse, Dietician, Registrar, RMO, Clinical Pharmacist	Not required
9. Report any suspected adverse drug reaction to the Therapeutic Goods Administration (TGA) (online via MiMs or TGA website) and document in patient medical record.	All clinicians	

2.0 Patient with current PAS Clinical Alert presents to Emergency Department or for a planned direct admission (non-ED presentation)

Responsibility	Emergency Department (ED)/inpatients	Outpatient areas
1. Print a copy of the PAS clinical alerts and attach to the front of the PAS Admission Registration.	Clerical Officer	Admission / Ward Clerk
2. Attach a red patient identification (ID) band to the patient if an allergy or ADR is identified.	Nurse, Clerical Officer	
3. Affix ' Adverse Drug Reaction ' alert sticker on all current medication charts for ADRs and complete the ADR/allergy section for all medication related clinical alerts.	Consultant, Registrar, RMO, Nurse, Clinical Pharmacist	

3.0 Updating or deleting a Clinical Alert on PAS

- An electronic MR002 form is to be completed by Medical/Clinical staff or Clinical Coder to submit updated information regarding a patient's clinical alert(s).
- Health Information System Support Office (PMI clerks) to print out a copy of the MR002 form and forward with the patient's medical record to SCGOPHCG Clinical Alert Committee for review.
- Following the committee's review and approval, HISSO will update PAS accordingly.

4.0 Clinical Alert Committee Review Process

Responsibility	Role
1. All new Clinical Alert MR002 forms are printed and brought to the SCGOPHCG Clinical Alert Committee.	HISSO
2. Committee to review appropriateness of Clinical Alert • If approved: ○ Complete 'Clinical Alert Committee Use Only' section of MR002 form. ○ Ensure form is sent to medical records for scanning into the Healthcare record • If not approved: ○ Complete 'Clinical Alert Committee Use Only' section on MR002 form. ○ Endorse across the MR002 form diagonally with 'Not Approved' and state the reason why not approved. ○ Provide PMI clerk with MR002 to update/inactivate alert on PAS. ○ Ensure form is sent to medical records for scanning into the Healthcare record ○ Notify Consultant, patient and GP via template letter where appropriate. • If confirmation of adverse drug reaction is required ○ eReferral is generated to immunology for follow up. ○ Accompanied by letters to notify submitting team and patient.	SCGOPHCG Clinical Alert Committee

Compliance, monitoring and evaluation

The following indicators are to be reported to the North Metropolitan Health Service Executive Director of Consumer Experience and Clinical Excellence (CECE) and the Department of Health Office of Safety and Quality:

- Percentage (%) (including numerator/denominator information) of alerts that were reviewed as appropriate to be entered onto PAS.
- Percentage (%) (including numerator/denominator information) of timeliness of clinical alert entry onto the PAS.
- Percentage (%) (including numerator/denominator information) of medical, nursing, pharmacy and clerical staff who have received education on clinical alerts.
- Health service has a policy document in place which includes governance arrangements, roles and responsibilities, and procedures.

- Health service has an established Clinical Alert Committee with the authority and resources to enable monitoring and evaluation.

The committee will also monitor:

- Average time for alerts to be uploaded onto PAS from generating MR002 form (date of form filled out to date entered onto PAS)
- Clinical Alert reports as determined by the committee
- Annual committee member attendance analysis
- An Annual Committee Effectiveness Evaluation Report.

Related Documents

[MP0053/17 WA Clinical Alert \(MedAlert\) Policy](#)
[Medical Alert/Clinical Alert notification form MR002 form](#)
[Clinical Alerts page on SCGOPHCG hub.](#)

References

ASCIA. (2016). [Information for health professionals: Food allergy clinical update](#). Adapted from Perth Children's Hospital Clinical Alert policy
 Australian Commission on Safety and Quality in Health Care. Standard 4: Medication Safety. [National Safety and Quality Health Service Standards](#).
[Australian Society of Clinical Immunology and Allergy \(ASCIA\) website](#). Accessed June 2018.
 Western Australian Therapeutic Advisory Group (2009). Medication Safety Alert. [Recording and Communication of Adverse Drug Reactions \(ADR\)](#)
 Western Australia Department of Health: [Clinical Alert Resources](#)

Authorisation & Version Control

Policy Sponsor	Deputy Director Clinical Services				
Policy Contact	SCGOPHCG Senior Pharmacist				
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Standards Applicable					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

This document can be made available in alternative formats on request for a person with a disability.

Appendix 1: Clinical Alert Codes

For the most up to date Med Alert / Clinical Alert Codes, please refer to page 2 of the [Medical Alert/Clinical Alert notification form MR002 form](#).

Appendix 2: Serious Adverse Drug Reactions for Inclusion on the PAS

Allergic Reactions for Inclusion (Drug and Non Drug Allergies e.g. Latex, Intravenous Contrasts, Chlorhexidine):

- Rash – if thought to be serious or severe, or accompanied by swelling of the whole body (not localised)
- Anaphylaxis or Anaphylactoid reactions
- Serum Sickness
- Angioedema - swelling of face, throat, neck, tongue
 - Bronchospasm, asthma, other breathing difficulties.

Other serious of life threatening reactions for inclusion:

- Agranulocytosis (e.g. clozapine)
- Extrapyramidal side effects (severe dystonia / laryngospasm) to antipsychotics
- Stevens Johnson Syndrome
- Toxic epidermal necrolysis
- Malignant hyperthermia
- Scoline apnoea or cholinesterase problem
- Neuroleptic Malignant Syndrome
- Hepatitis or Nephritis
 - Other – must be deemed serious and life-threatening/causing significant harm.

Adverse drug reactions that are not deemed Clinical Alerts/Med Alerts:

- **Non-dose Related Reactions** – Unpredictable and uncommon side-effects not related to pharmacological action, with a low mortality rate (e.g. Timolol causing depression, Lithium induced neutropenia)
- **Time-related Reactions** – Uncommon reactions which are usually dose-related, and occur sometime after the use of the drug (e.g. Tardive dyskinesia secondary to antipsychotic medications)
- **Dose Related Reactions** – Predictable side-effects related to pharmacological action of medications (e.g. moderate extrapyramidal side-effects to antipsychotic medications, excessive nausea and vomiting with opioids, vancomycin causing Red Man Syndrome).

Mild to moderate side-effects or unknown reactions are not to be recorded as adverse drug reactions including:

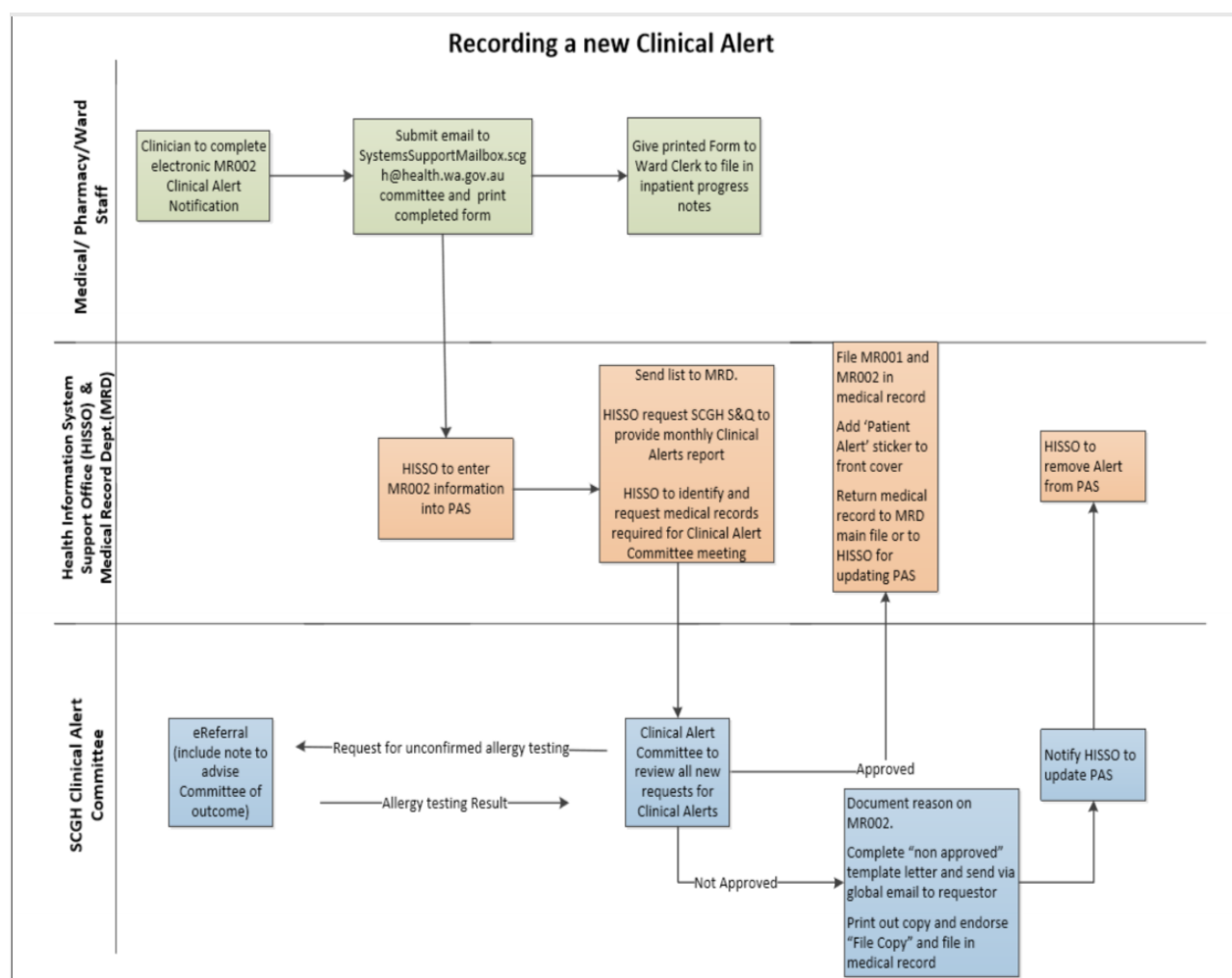
- Mild diarrhea, nausea and mild vomiting, itch, hay fever / blocked nose, local swelling or pain

Non-serious adverse reactions to non-drug allergens (e.g. bee stings, grasses) are not to be entered on the PAS as a clinical alert but should be recorded in the medical record.

Appendix 3: How to Initiate a Clinical Alert

Please refer to the '[How to Initiate a Clinical Alert](#)' on the SCGOPHCG hub for the latest version.

Appendix 4: Management of Clinical Alerts at SCGH



Appendix 5: Adverse Drug Reaction Information brochure for Consumers

A [print ready version](#) can be found on the Medication Safety Page on the SCGOPHCG hub.

Appendix 6: Dietary Allergy/Food Allergy

Dietary (Food) allergy is deemed a Clinical Alert (which must be differentiated from a food intolerance which is **not** a Clinical Alert).

A **food allergy** is an abnormal immune mediated reaction to ingested food, resulting in clinical symptoms. Reactions can occur after eating a small amount of food. Food allergy affects 10% of infants <1 year, 4-8% of children <5 years and up to 2% of the adult population in Australia. It is the responsibility of the treating medical team to ensure a Clinical Alert is recorded for patients with a food allergy.

A **food intolerance** does not involve the immune system and does not cause severe life threatening allergic reactions (known as anaphylaxis) and should not be listed as a clinical alert (see Clinical Alert definition). Food intolerance reactions are dose related and are often delayed. Symptoms may include headaches or gastrointestinal symptoms after eating.

Food Allergies to be reported as a Clinical Alert include:

- Anaphylaxis or anaphylactoid reaction

- Swelling of face, lips, eyes, tongue or throat
- Flushing or hives/welts on the skin
- Tingling mouth
- Severe abdominal pain, severe vomiting, severe diarrhoea
- Difficult /noisy breathing
- Difficulty talking and / or hoarse voice
- Wheeze or persistent cough
- Persistent dizziness and/or collapse
- Pale and floppy (in young children)
- Acute onset of hypotension, severe breathing difficulty, bronchospasm or upper airway obstruction where anaphylaxis is considered possible.

There are nine common allergens in Australia specified by the Australasian Society of Clinical Immunology and Allergy (ASCIa).

These are wheat, egg, soy, cow's milk (diary), peanuts, tree nuts, fish, shell fish (Crustacean), and sesame.

These foods cause around 90% of food allergic reactions in Australia. Food labels are required by law to declare if the food or beverage contains any of these common nine allergens. (Note: The legislation identifies gluten, not wheat). Outside of the nine common food allergens, any other food can cause an allergic reaction.

A food allergy Clinical Alert (DOther10), must include in the comments field the words either 'unconfirmed' or 'confirmed' depending on what follow up has been completed in determining the association between the allergy symptoms and the suspected food allergen.

A 'confirmed' food allergy must be diagnosed by an immunology or allergy specialist. When a food allergy challenge has been completed, the hospital (or HSP for smaller sites) must have a system in place to document de-labelling of a food allergy including removal of a food allergy Clinical Alert.

Appendix 7: WA Clinical Alert – Glossary

When the history of an alert is uncertain, but there is information to suggest an alert may be possible, the term 'unconfirmed' should be added to the free text information when submitting an alert rather than 'history of'.

There should be a process of prompting unconfirmed allergy related clinical alerts (angioedema/ anaphylaxis) to immunology for review to confirm allergy status. Issue with this is that some sites are not receiving feedback following immunology testing. Plan to put alert on system and remove if confirmed not allergy related by immunology.

The WA Clinical Alert Business User Group is the governing body for all changes made to Clinical Alerts and relevant policy.

Any enquires for removal, modification or additional alerts must be directed to the secretariat of the WA Clinical Alert Business Group (safetyandquality@health.wa.gov.au).

Clinical alerts entered into webPAS have a text field with the capacity for 200 character entry to provide the description of the alert.

1. Authorisation process

- The designated local committee / dedicated position (depending on size of institution - ideally a medical officer or clinical pharmacist (if drug related)) responsible for governance of clinical alerts for each site will review all anaesthetic,

medical and drug alert queries in a timely manner, with the exclusion of anaesthetic and specified alerts.

- Some Clinical Alerts (specified alerts) can be entered without co-authorisation (i.e. organ transplant, heart valve replacements, pacemaker or other implanted devices when inserted during the admission being coded). Alerts that meet this criterion should be decided by the governing body within the hospital.

2. Data entry and file form in medical record

- Once the clinical alert has been approved for entry onto PAS, forward form with the medical record (if requested) to clinical coding to be entered onto the system.

If the proposed clinical alert is not approved for entry onto PAS

- If the alert is not deemed to be a clinical alert (i.e. serious/life threatening issue) but still requires to be captured in the patient's medical record (such as mild to moderate side-effects to a drug) the position responsible for approval of alerts should ensure the alert (medical, anaesthetic or drug alert information) is documented on inside cover of the health record, the Patient Alert Form (MR ALERT1) or in the digital medical record if not already documented.
- Cross the Clinical Alert/ Med Alert Notification forms through with two diagonal lines and state the reason why not approved. The MR ALERT 2 form should still be filed as a record of proposed Clinical Alert with reason why it was not approved.

Removal of alerts from PAS

- If an alert is entered in error or is no longer relevant it needs to be removed from the PAS to reduce the risk to the patient.

When the alert codes are being reviewed and need to be updated (new ones added or obsolete ones removed) then a change request is raised by the committee to update the PAS, and the relevant communications sent to stakeholders regarding the change.

Anaesthetic Alerts		
Difficult Intubation/ Difficult Airway and Oxygenation	Specify type including details of limited neck movement, describe grade of difficulty, device used and bag mask ventilation (BMV) status.	A.Other1
Anaesthetic Drug Reaction	Specify anaesthetic medication, reaction details and date that reaction occurred. (Anaphylactic reaction to anaesthetic drug/s should be recorded under D02.03 Anaphylaxis – see alert below).	A02.01
Malignant Hyperthermia	Specify anaesthetic agent involved and date of reaction. Malignant hyperthermia (MH) manifests clinically as a hypermetabolic crisis when an MH-susceptible (MHS) individual is exposed to a volatile anaesthetic or suxamethonium.	A02.04
Sleep Apnoea	Sleep apnoea syndromes are characterized by the absence of airflow with or without ventilatory effort during sleep. Only report severe sleep obstructive sleep apnoea and those requiring Continuous Positive Airway Pressure (CPAP).	A03.01
Drug/Dietary Reaction		
Life-long Anticoagulant	Specify anticoagulant and indication. Not related to heart valve replacement (as this is identified as Heart Valve Replacement alert - M.Other1, see alert below)	D.Other1
Serious Drug Reactions	Specify drug, reaction details and date that reaction occurred. A serious adverse drug reaction is defined reaction that may lead to a life-threatening event and has an absolute or relative	D.Other2

	contraindication to repeat administration of the drug, or where adrenaline autoinjector (e.g. Epipen®) is required. <u>Allergic reactions for inclusion:</u> (Drug and Non Drug Allergies e.g. Latex, Intravenous Contrasts, Chlorhexidine): • Rash – if thought to be serious or severe, or accompanied by swelling of the whole body (not localised). • Anaphylaxis or Anaphylactoid reactions. • Serum Sickness. • Angioedema - swelling of face, throat, neck, tongue. • Bronchospasm, asthma, other breathing difficulties. <u>Other serious or life threatening reactions for inclusion:</u> • Agranulocytosis (e.g. clozapine). • Extrapyrimal side effects (severe dystonia / laryngospasm) to antipsychotics. • Stevens Johnson Syndrome. • Toxic epidermal necrolysis. • Malignant hyperthermia. • Scoline apnea or cholinesterase problem. • Neuroleptic Malignant Syndrome. • Hepatitis or Nephritis. • Other – must be deemed serious and life-threatening/causing significant harm. <u>Adverse drug reactions that are NOT deemed Clinical Alerts/Med Alerts:</u> • Non-dose Related Reactions – Unpredictable and uncommon side-effects not related to pharmacological action, with a low mortality rate (e.g. Timolol causing depression, Lithium induced neutropenia). • Time-related Reactions – Uncommon reactions which are usually dose-related, and occur sometime after the use of the drug (e.g. Tardive dyskinesia secondary to antipsychotic drugs). • Dose Related Reactions – Predictable side-effects related to pharmacological action of drugs (e.g. moderate extrapyramidal side-effects to antipsychotic drugs, excessive nausea and vomiting with opioids, vancomycin causing Red Man Syndrome). • Mild to moderate side-effects or unknown reactions are not to be recorded as a clinical alert but should be documented in the medical record. Examples of these include: - mild diarrhoea, nausea and mild vomiting, itch, hay fever / blocked nose, local swelling or pain. - Non-serious adverse reactions to non-drug allergens (e.g. bee stings, grasses).	
Angioedema	Specify drug and date that reaction occurred. Angioedema is self-limited, localised subcutaneous (or submucosal) swelling, which results from extravasation of fluid into interstitial tissues. Angioedema may occur in isolation, accompanied by urticaria, or as a component of anaphylaxis.	D02.02
Anaphylaxis	Specify drug, reaction details and date that reaction occurred.	D02.03

	Anaphylaxis is an acute, potentially life-threatening, multisystem syndrome caused by the sudden release of mast cell mediators into the systemic circulation. Symptoms may include: • Respiratory compromise (e.g. dyspnoea, wheeze, persistent cough, difficult/noisy breathing, stridor, hypoxemia) • Reduced blood pressure or associated symptoms (e.g. persistent dizziness, collapse, hypotonia, syncope, incontinence) • Involvement of the skin-mucosal tissue (e.g. generalized hives, itch-flush, swollen lips-tongue-uvula). • Swelling/tightness in throat • Difficulty talking and/or hoarse voice • Persistent gastrointestinal symptoms and signs (e.g. crampy abdominal pain, vomiting). • Pale and floppy (young children)	
Anti-venom given	Specify anti-venom given and date administered	D05.01
Chronic Steroids	Specify condition requiring chronic steroid treatment. All patients on daily continuous steroid therapy.	D10.01
Clozapine	Specify date patient initiated on clozapine therapy Alert is to remain on system regardless of whether currently taking clozapine or not. Review clozapine monitoring system for current status. Contact pharmacy for further information.	D11.01
Serious Dietary/Food Reactions	Specify Dietary/Food allergen and reaction. Specify if allergy requires adrenaline autoinjector (e.g. Epipen®) Refer to Clinical Alert Policy (Appendix 6) A <u>food allergy</u> is an abnormal immune mediated reaction to ingested food, resulting in clinical symptoms. Reactions can occur after eating a small amount of food. <u>Food Allergies</u> to be reported as a clinical alert include: • Anaphylaxis or anaphylactoid reaction • Swelling of face, lips, eyes, tongue or throat • Flushing or hives/welts on the skin • Tingling mouth • Severe abdominal pain, severe vomiting, severe diarrhoea • Difficult /noisy breathing • Difficulty talking and / or hoarse voice • Wheeze or persistent cough • Persistent dizziness and/or collapse • Pale and floppy (in young children) • Acute onset of hypotension, severe breathing difficulty, bronchospasm or upper airway obstruction where anaphylaxis is considered possible. • If allergy requires adrenaline autoinjector (e.g. Epipen®)	D.Other10
Medical Conditions		
Heart Valve Replacement	Specify site and type (e.g. mechanical or porcine). Also add "Life Long Anticoagulant"	M.Other1
Implanted Devices	Specify device and site Devices include Permanent Pacemaker (PPM), VVT Ventricular Tachycardia PPM, neurostimulator, continuous subcutaneous insulin pumps, heart monitor or defibrillator, ventricular shunt, intrathecal pump, implanted hearing devices (e.g. Cochlear, BAHA implants)	M.Other3
Other Medical Conditions	Specify condition	M.Other4

	If medical condition not currently listed as an approved clinical alert. Must be related to a diagnosis which has the potential to be of critical, life-threatening importance to a patient's management during the first 24 hours of their admission to hospital and assumes that the patient is not always capable of communicating such information. <i>This alert code must not be used as a 'registry' to identify patients that do NOT have life threatening conditions e.g. patient having cancer treatment or dialysis.</i> <i>These clinical alerts raised into the PAS will be reviewed by the Clinical Alert Committee of the organisation within which it was raised as well as the Clinical Alert Business User Group for consideration of a new clinical alert code.</i>	
Streptokinase Therapy	Specify site, hospital and date of administration Usage in Respiratory medicine for complicated parapneumonic effusions and empyema generally administered by intercostal catheter. Anti-streptokinase antibodies remain elevated for up to 7.5 years after treatment, suggesting that a suboptimal response and an allergic reaction may occur with retreatment many years after the first exposure.	M.01.02
Bleeding Disorder	Specify condition Clinician to indicate classification of bleeding disorder that requires clinical alert notification. (e.g. Haemophilia, von Willebrand disease, Christmas Disease).	M.01.03
Sickle Cell Anaemia	Sickle cell disease (SCD) refers to any one of the syndromes in which the sickle mutation is co-inherited with a mutation at the other beta globin allele that reduces or abolishes normal beta globin production. Sickle cell anaemia (homozygous sickle mutation) produces a chronic, compensated haemolytic anaemia that may be punctuated with episodes of acute worsening.	M01.04
Severe Arrhythmia	Specify type of arrhythmia and treatment including; • Ventricular tachycardia • Supraventricular tachycardia • Ventricular fibrillation • Wolff Parkinson White Syndrome	M02.02
Hypopituitary	Hypopituitarism is defined as deficient secretion of one or more pituitary hormones because of pituitary or hypothalamic disease (e.g. corticotrophin (ACTH), thyrotropin, gonadotropin, growth hormone, or prolactin)	M03.07
Addison's Disease	Primary adrenal insufficiency – may result in adrenal crisis/shock due to mineralocorticoid and glucocorticoid deficiency.	M03.08
Porphyria	Specify class/type The porphyrias are metabolic disorders caused by altered activities of enzymes within the haeme-biosynthetic pathway. Porphyrias can be classified as either acute or cutaneous, reflecting the primary clinical manifestations. The porphyrias are also classified as hepatic or erythropoietic, depending on whether the site of initial production and accumulation of pathway intermediates is the liver or the bone marrow. <u>Acute porphyrias</u> — The acute porphyrias (ie, those with neurovisceral manifestations) include the following: • ALA dehydratase porphyria (ADP) • Acute intermittent porphyria (AIP) • Hereditary coproporphyria (HCP) • Variegate porphyria (VP)	M04.01

Neuroleptic Malignant Syndrome	Specify neuroleptic agent involved and date reaction occurred Neuroleptic malignant syndrome (NMS) is a life-threatening neurologic emergency associated with the use of neuroleptic agents and characterized by a distinctive clinical syndrome of mental status change, rigidity, fever, and dysautonomia.	M04.04
G6PD Deficiency	Glucose-6-phosphate dehydrogenase (G6PD) deficiency is an inherited disorder caused by a genetic defect in the red blood cell (RBC) enzyme G6PD, which generates nicotinamide adenine dinucleotide phosphate (NADPH) and protects RBCs from oxidative injury. G6PD deficiency is the most common enzymatic disorder of RBCs.	M04.05
Thalassaemia	Thalassemia is a group of disorders in which the normal ratio of alpha globin to beta globin production is disrupted due to a disease-causing variant in one or more of the globin genes. This can cause destruction of red blood cell precursors in the bone marrow (ineffective erythropoiesis) and circulation (haemolysis). As a result, affected individuals have variable degrees of anaemia and extramedullary haematopoiesis, which in turn can cause bone changes, impaired growth, and iron overload.	M04.06
Severe Epilepsy	Only report when diagnosis documented as intractable / recurrent seizures.	M05.01
Myaesthesia Gravis	Myasthenia gravis is a disorder of neuromuscular transmission involving a fluctuating degree and variable combination of weakness in ocular, bulbar, limb, and respiratory muscles.	M.05.02
Fabricated Illness	Fabricated illness (or factitious disorder imposed on self) is characterized by falsified general medical or psychiatric symptoms. Patients deceptively misrepresent, simulate, or cause symptoms of an illness and/or injury in themselves, even in the absence of obvious external rewards. May also include 'Munchausen's by Proxy'.	M06.01
Difficult X-Match	Antibody cross match – to be confirmed by PathWest/Haematology. Haematology documentation required. Not applicable for in Rhesus positive (+) or negative (-) pregnant mothers.	M09.01
Organ Transplant	Specify organ transplanted, and hospital and date if available.	M10.06
Advance Health Directive (AHD)	Specify "AHD exists" only It is expected that if this flag is raised that the clinician will review the patient's medical record to determine if the AHD was raised at the hospital of admission and/or initiate a discussion with patient/carer as to where the AHD was filed and the currency of the content. Refer to Appendix 10 of Clinical Alert Policy	M11.01
Asplenia	Specify whether hyposplenia (spleen intact but not functioning) or, partial or full removal of spleen, and immunisation status where available.	M12.01